

1. Administrative & Study Identification

Full Study Title: A Randomized, Double-Blind, Placebo-Controlled, Parallel-Group, Multi-Center, Phase II Study to Evaluate the Efficacy, Safety, and Tolerability of Once-Daily R07795081 Administered for 38 Weeks to Participants With Obesity or Overweight With at Least One Weight-Related Comorbidity **Short Title/Acronym:** A Study Evaluating the Effect of Daily Oral R07795081 in Participants With Obesity or Overweight With at Least One Weight-Related Comorbidity **Protocol Number:** BP45702 **NCT Number:** NCT07081958 **Sponsor Name:** Hoffmann-La Roche **Investigational Product:** R07795081 (also known as CT-996, RG6652) **Phase of Development:** Phase II **Medical Monitor:** Hoffmann-La Roche **Clinical Operations Lead**

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy, tolerability, and safety of R07795081 for chronic weight management in adult participants with obesity or overweight with at least one weight-related comorbidity, but without diabetes mellitus. **Secondary Objectives:** To establish the optimal dose range for further phase III development, evaluate reductions in body weight, and assess improvements in weight-related comorbidities over the treatment period.

2.2 Background & Rationale To address the clinical need for effective, non-invasive oral treatments for chronic weight management. The study evaluates R07795081 against placebo to confirm safety and tolerability before wider testing, targeting adults with elevated BMI (≥ 27.0 kg/m²) who struggle to lose weight through traditional diet and exercise modifications alone.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind **Randomization:** Randomized, Parallel-group

3.2 Planned Enrollment & Duration Target N: 340 **Number of Sites:** Multi-center (Multiple sites across the United States including CA, CT, FL, GA, IL, NY, NC, and PA) **Estimated Study Start:** July 29, 2025 **Estimated Primary Completion:** April 23, 2026 **Estimated Study Completion:** July 17, 2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Body mass index (BMI) greater than or equal to ≥ 30.0 kg/m²; or BMI ≥ 27.0 kg/m² and <30.0 kg/m² with at least one weight-related comorbidity, such as hypertension, dyslipidemia, diagnosis of obstructive sleep apnea, or cardiovascular disease. 3. History of ≥ 1 self-reported unsuccessful diet/exercise effort to lose body weight. 4. A stable body weight for the 3 months prior to screening (maximum 5% body weight gain and/or loss).

4.2 Exclusion Criteria 1. Have a history or diagnosis of any type of diabetes mellitus (Type 1 diabetes [T1D], T2D, or rare forms of diabetes). 2. Have obesity induced by other endocrinologic disorders (e.g., Cushing's syndrome) or diagnosed monogenetic or syndromic forms of obesity (e.g., melanocortin-4 receptor deficiency or Prader-Willi Syndrome). 3. Participation in unbalanced/extreme diets or in an organized weight reduction program within 3 months of the screening visit or planning to engage in such diets or programs during the study. 4. Prior or planned surgical treatment for obesity. 5. Have any significant active or unstable major depressive disorder (MDD), severe psychiatric disorder, or suicide attempt within the last 1 year. 6. Have a known, clinically significant gastric emptying abnormality. 7. Have any of the following cardiovascular conditions within 3 months prior to screening: Acute myocardial infarction; Cerebrovascular accident (stroke)/transient ischemic attack; Unstable angina; Hospitalization due to congestive heart failure. 8. Have poorly controlled hypertension, untreated renal artery stenosis, or evidence of labile blood pressure including symptomatic postural hypotension.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: R07795081 administered once daily. Route of Administration: Oral. Duration of Treatment: 38 weeks.

5.2 Concomitant & Prohibited Therapy Permitted: Stable management medications for existing comorbidities (e.g., hypertension, dyslipidemia). Prohibited: Concurrent participation in another weight reduction program, extreme diets, and medications strictly indicated for diabetes management (GLP-1 inhibitors etc.).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, BMI check, Psychiatric evaluations (PHQ-9, C-SSRS)
Baseline	Day 0	Randomization, First Dose, Vital Signs	Interim Weeks 4-34 Safety Labs, Efficacy Assessment

(Body Weight Monitoring), Education | | **End of Study** | Week 38 |
Final Vital Signs, Exit Interview, IP Accountability |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: The primary efficacy assessment is the percentage change in body weight from baseline to Week 38. **Measurement Method:** Clinical assessment evaluated alongside safety/tolerability measures.

7.2 Secondary & Exploratory Endpoints - Proportion of participants achieving $\geq 5\%$, $\geq 10\%$, or $\geq 15\%$ weight loss at Week 38. - Change from baseline in weight-related comorbidity biomarkers (e.g., blood pressure, lipids). - Safety and psychiatric assessments utilizing Patient-Reported Outcomes (PHQ-9 for depression; C-SSRS for suicidality).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard clinical collection coupled with specialized patient-reported outcome questionnaires (PHQ-9 and C-SSRS). **Serious Adverse Events (SAEs):** Evaluated and recorded actively throughout the study period. **Data Monitoring Committee (DMC):** An independent Data Monitoring Committee (DMC) of scientists is engaged to monitor the safety and scientific integrity of the trial, with the authority to recommend study termination if participants are harmed.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy; Safety Set for AE/SAE reviews. **Sample Size Justification:** Sized accordingly for a Phase II dose-range-finding parallel-group design to detect clinical efficacy over placebo. **Handling of Missing Data:** Standard handling procedures for early discontinuation and missing assessments utilizing appropriate multiple imputation statistical modeling.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Continuous site monitoring, coupled with independent DMC oversight to ensure subject safety. **Audit Trail:** Electronic case report forms (eCRF) utilized for secure data storage, lock, and clarity mechanisms.

11. Study Identifiers & Governance

Primary ID: BP45702 **Registry Number:** NCT07081958 **Sponsor/Lead Organization:** Hoffmann-La Roche **Study Title:** A Study Evaluating the Effect of Daily Oral R07795081 in Participants With Obesity or Overweight With at Least One Weight-Related Comorbidity **Official Regulatory Title:** A Randomized, Double-Blind, Placebo-Controlled, Parallel-Group, Multi-Center, Phase II Study to Evaluate the Efficacy, Safety, and Tolerability of Once-Daily R07795081 Administered for 38 Weeks to Participants With Obesity or Overweight With at Least One Weight-Related Comorbidity

12. Clinical Framework (Obesity/Overweight Specific)

Primary Condition: Obesity or Overweight with at least one comorbidity **Diagnosis Threshold:** BMI ≥ 30.0 kg/m² OR BMI ≥ 27.0 kg/m² with weight-related comorbidity **Medical Methodology:** Pharmacological intervention combined with dietary and exercise efforts **Optimization Target:** Chronic weight management and sustained weight reduction

13. Study Procedures & Methodology

Management Pathway: Oral pharmacological pathway via 38-week continuous dosing **Education Protocol (HCP):** Site initiation visits and standardized trial procedure training **Education Protocol (Patient):** Guidance on avoiding extreme unorganized diets while maintaining a stable weight framework **Quality Audit Mechanism:** Active independent data monitoring by the DMC alongside continuous clinical tracking

14. Observation & Follow-Up Schedule

Total Duration: 38 Weeks of treatment phase **Onsite Visit Cadence:** Regular periodic intervals to capture vital signs and PRO data **Remote Monitoring Frequency:** Routine queries tracking psychiatric risk tools (PHQ-9/C-SSRS) **Checklist Review Interval:** Continuous throughout the 38-week protocol lifecycle

15. Sign-off & Approval

	Role		Name		Signature		Date			:---		:---		:---		:---	
	Principal Investigator																
_____	[DD-MMM-YYYY]																
	Clinical Operations Lead																
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	Lead Statistician																
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